

Patient ID: WimberleyPeriod: December 7 to January 6, 2024Report Date: January 7, 2024Coverage: 478/726h (66%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Dec 07 to Jan 06

■ HR ■ Breathing

Elevated Breathing Dec 09 to Dec 11 (3d)	Elevated Breathing Dec 26 to Dec 28 (3d)	High Breathing Dec 09 to Dec 17 (9d)	High Breathing Dec 27 to Dec 30 (4d)	Very High Breathing Dec 07 to Dec 14 (8d)	Very High Breathing Dec 28 to Dec 31 (4d)
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Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	62 bpm	52 bpm	71 bpm
Breathing	20 brpm	6 brpm	30 brpm

Limited data in the final 24h (16/24h); values reflect available readings.

Last 24 hours: one very high breathing episode in the afternoon; one elevated breathing episode in the morning.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jan 06	8 AM to 9 AM	2h	2	Elevated Breathing	29	9	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jan 06	1 PM to 2 PM	1h	1	Very High Breathing (brief)	54	45	30	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

53 episodic events Detected, spanning 58h total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
53	58h	High Breathing	2	66%

Major Findings: Sustained high breathing (avg 34, peak 60)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Dec 07	9 PM to 10 PM	11h	10	Very High Breathing	49	8	60	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 28	4 PM to 5 PM	4h	4	Very High Breathing	48	14	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 09	6 PM to 8 PM	17h	14	High Breathing	34	3	50	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 27	2 PM to 3 PM	4h	4	High Breathing	35	9	51	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Dec 09	12 PM to 1 PM	2h	2	Elevated Breathing	27	9	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
Dec 26	9 PM to 10 PM	6h	6	Elevated Breathing	28	9	38	Check SpO2 and lung sounds; assess for fluid overload or early infection

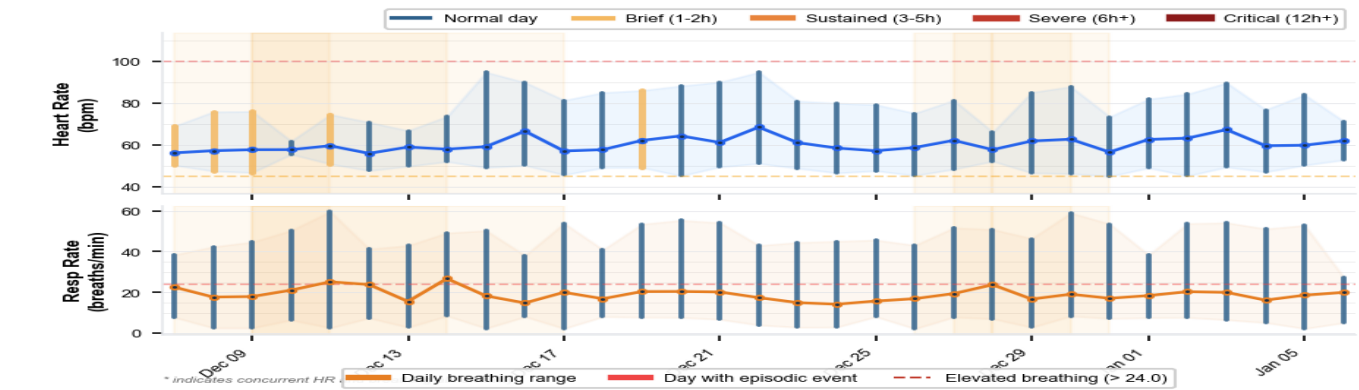
The P5 to P95 heart rate spread was 21 bpm (52 to 73), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- High Breathing (avg 34 brpm, peak 50 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 49 brpm, peak 60 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 28 brpm, peak 38 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 6 unstable phases with 21 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

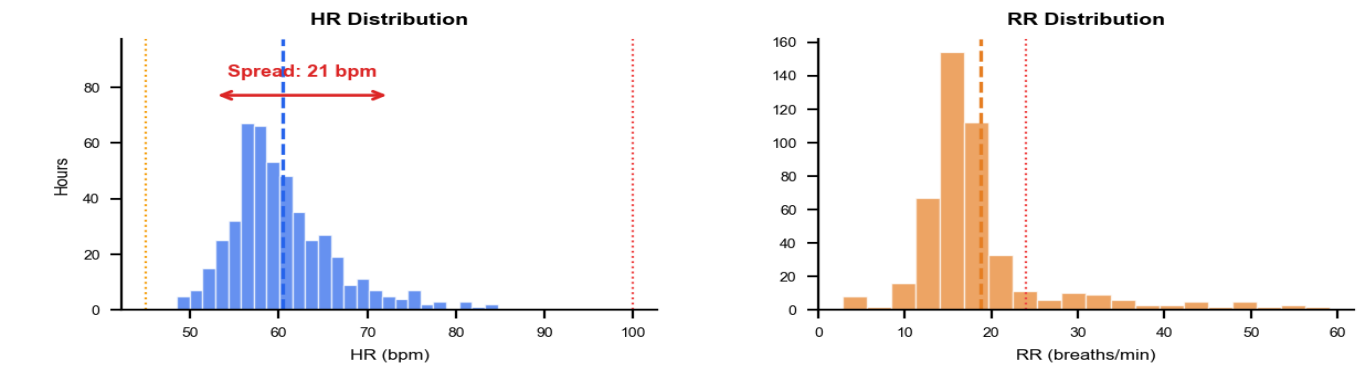
Clinical Pattern Observations:

- 1. **High variability:** HR spread 20 bpm (52 to 72).
- 2. **Breathing variability:** RR spread 27 brpm (11 to 38).
- 3. **Clustered pattern:** Episodic events on 5 of 31 days (16%).

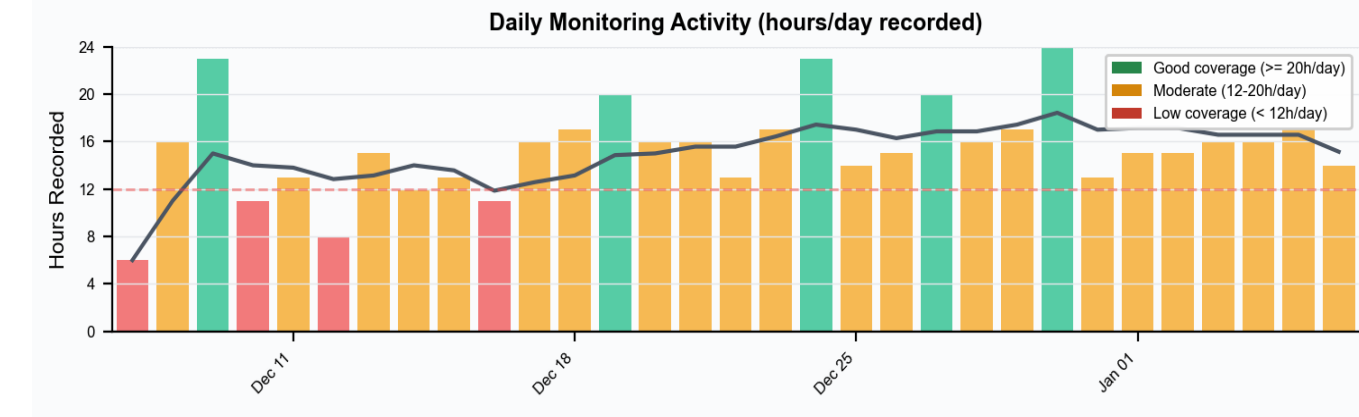


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
 - Low HR < 45 bpm
 - Elevated HR > 95 bpm
 - High HR > 100 bpm
 - Very High HR > 110 bpm
 - Elevated Breathing > 24 brpm
 - High Breathing > 30 brpm
 - Very High Breathing > 40 brpm
- Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.